

Drug Reaction with Eosinophilia and Systemic Symptom (DRESS) induced by carbamazepine: a case report and literature review.

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Drug-induced hypersensitivity or Drug Reaction with Eosinophilia and Systemic Symptom (DRESS) is a severe adverse drug-induced reaction. Diagnosing DRESS is challenging due to the diversity of cutaneous eruption and organs involved. Most of the aromatic anticonvulsants, such as phenytoin, phenobarbital, and carbamazepine, can induce DRESS. Culprit drug withdrawal and corticosteroids constituted the mainstay of DRESS treatment. We describe a 6 year-old boy who presented fever and rash 4 weeks after starting carbamazepine. Investigation revealed leukocytosis, atypical lymphocytosis, and elevated serum transaminases. The diagnosis of DRESS syndrome was made, Carbamazepine was stopped and replaced initially by Clobazam and by Valproic acid after discharge, no systemic corticotherapy was prescribed. Symptoms began to resolve within two weeks, and by one month later her laboratory values had returned to normal. The aim of this work is to raise awareness general practitioner and pediatricians to suspect Dress syndrome in patients who present with unusual complaints and skin findings after starting any antiepileptic drug.

Acute renal failure secondary to drug-related crystalluria and/or drug reaction with eosinophilia and systemic symptom syndrome in a patient with metastatic lung cancer.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) or drug-induced hypersensitivity is a severe adverse drug-induced reaction. Aromatic anticonvulsants, such as phenytoin, phenobarbital, and carbamazepine, and some drugs, can induce DRESS. Atypical crystalluria can be seen in patients treated with amoxycillin or some drugs and can cause acute renal failure. We describe a 66-year-old man who presented fever and rash and acute renal failure three days after starting amoxycillin. He was also using phenytoin because of cerebral metastatic lung cancer. Investigation revealed eosinophilia and atypical crystalluria. The diagnosis of DRESS syndrome was made, amoxicillin was stopped, and dose of phenytoin was reduced. No systemic corticosteroid therapy was prescribed. Symptoms began to resolve within three to four days. The aim of this paper is to highlight the importance of microscopic examination of urine in a case with acute renal failure and skin lesions to suspect DRESS syndrome.

DRESS syndrome: cerebral vasculitic-like presentation.

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DRESS (drug reaction with eosinophilia and systemic symptoms) syndrome is a severe adverse drug-induced reaction. It manifests with pyrexia, eosinophilia, and lymphadenopathy, with multiple organ involvement, mainly the skin, liver, and kidneys. The purpose of this article is to demonstrate that DRESS syndrome can be associated with cerebral manifestations, a concept not well known in the neuroradiological literature. We describe three cases of DRESS syndrome associated with cerebral vasculitic-like lesions and realize a review of the literature to demonstrate that this association represents a very rare entity. Acute ischemic lesions were found among two patients. In all cases, perivascular enhancement was present. Magnetic resonance angiography (MRA) sequence was normal.

Although no cerebral biopsy was performed, this enhancement pattern is strongly suggestive of a vasculitic process associated with DRESS syndrome. Diagnosis of cerebral vasculitic-like associated lesions must be considered in patients with DRESS syndrome since it can be reversed completely by withdrawing the causal medication and instigating corticosteroid treatment in a timely fashion.

Drug reaction with eosinophilia and systemic symptoms (DRESS) and multiple organ dysfunction syndrome (MODS): one more reason for a new effective treatment against leishmaniasis.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome is a severe drug-induced reaction associated with eosinophilia and systemic manifestations. Anticonvulsants, sulfonamides, and antivirals are the most related and described drugs in DRESS syndrome. We present a case of severe multiple organ dysfunction syndrome (MODS) with the risk of death associated with DRESS syndrome due to antileishmanial pentavalent antimonial drug and its simultaneous toxicity. Consequently, a comprehensive review of the main clinical problems and comparative discussion of both clinical conditions was made. The overlap of DRESS syndrome and antileishmanial pentavalent antimonial drug toxicity can be life-threatening. Both conditions represent a true clinical, diagnostic, and therapeutic challenge. We exposed specific clinical and laboratory results with rare occurrence. Any physician and dermatologists should keep in mind the broad spectrum of clinical manifestations and laboratory findings associated with the use of pentavalent antimonial drugs. The clinical suspicion, an early diagnosis, and aggressive treatment are essential to prevent complications and death.

Drug rash with eosinophilia and systemic symptoms (DRESS syndrome).

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A variety of different drugs has been associated with a rather distinct clinical illness characterised by skin rash, fever and single or multiple organ involvement, which starts within 8 weeks after the initiation of the therapy. There is no consensus in the denomination of this apparently distinct clinical entity so that the acronym DRESS (drug rash with eosinophilia and systemic symptoms) has been recently proposed to focus thinking on this particular and probably under-recognised subset of drug reaction. The aim of this review is to point to the new perspectives regarding the diagnosis, the physiopathology and the management of DRESS syndrome.

DRESSed for distress: a case of allopurinol-induced DRESS syndrome.

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DRESS (Drug Reaction with Eosinophilia and Systemic Symptoms) syndrome is a severe systemic drug reaction characterized by a latent period of several weeks following the initiation of drug therapy. Among the most well-known causative agents is allopurinol, commonly prescribed for managing asymptomatic gout. Allopurinol-induced DRESS syndrome is associated with high mortality rates and significant

long-term sequelae. This report details the case of an elderly female patient who presented with an extensive rash covering her trunk and extremities which was concurrent with her use of allopurinol. The condition progressed to renal impairment but showed significant improvement upon cessation of the drug and administration of high-dose corticosteroids. This case aims to shed light on one of the most underrecognized types of systemic drug reactions, hoping to raise awareness about this rare yet serious complication of one of the most widely prescribed drugs.

Drug Reaction With Eosinophilia and Systemic Symptoms: A Diagnostic Dilemma.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) is an adverse reaction to medications such as sulfonamide-containing antibiotics, anticonvulsants, vancomycin, and non-steroidal anti-inflammatory drugs (NSAIDs). It typically presents with a characteristic rash, eosinophilia, and visceral organ failure. Patients who do not present with characteristic features of DRESS are at risk for delayed diagnosis and treatment. Early diagnosis of DRESS is imperative in preventing unfavorable outcomes such as multi-organ involvement and death. This case report presents the case of a patient who was diagnosed with DRESS but did not display a classic presentation.

Drug-induced hypersensitivity syndrome/drug reaction with eosinophilia and systemic symptoms. Part I. Epidemiology, pathogenesis, clinicopathological features, and prognosis.

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Drug-induced hypersensitivity syndrome (DiHS), also known as drug reaction with eosinophilia and systemic symptoms (DRESS), is a severe cutaneous adverse reaction (SCAR) characterized by an exanthem, fever, and hematologic and visceral organ involvement. Anticonvulsants, antibiotics, and allopurinol are the most common triggers. The pathogenesis involves a complex interplay between drugs, viruses, and the immune system primarily mediated by T-cells. DiHS/DRESS typically presents with a morbilliform eruption 2-6 weeks after drug exposure, and is associated with significant morbidity, mortality, and risk of relapse. Long-term sequelae primarily relate to organ dysfunction and autoimmune diseases. Part I of this continuing medical education activity on DiHS/DRESS provides an update on epidemiology, novel insights into pathogenesis, and a description of clinicopathological features and prognosis.

Oxacillin-Induced Drug Reaction with Eosinophilia and Systemic Symptoms (DRESS).

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BACKGROUND Drug reaction with eosinophilia and systemic symptoms (DRESS) is an idiosyncratic life-threatening reaction comprised of fevers, rash, and leukocytosis with eosinophilia. Though characteristically associated with leukocytosis, there are rare case reports of DRESS-induced agranulocytosis. DRESS is most frequently caused by antiepileptic medications; however, it has very

rarely been reported in relation to oxacillin. We describe a case of oxacillin-induced DRESS associated with agranulocytosis. CASE REPORT A 52-year-old male was admitted for an epidural abscess secondary to oxacillin-sensitive *Staphylococcus aureus*, for which an extended course of oxacillin and rifampin was initiated. On day 22 of therapy, the patient developed a fever of 38.7°C (101.6°F) with rigors. His complete blood cell count revealed new leukopenia ($1.8 \times 10^3/\mu\text{L}$) with 16% eosinophils and 3% atypical lymphocytes. Antibiotics were transitioned from oxacillin and rifampin to vancomycin, cefepime, and rifampin for presumed sepsis of unclear etiology. On day 23, he was noted to have a pruritic erythematous blanching papular rash on his chest, trunk, neck, and left upper extremity. Infectious workup was unrevealing, and his fever curve up-trended to 39.3°C (102.7°F) with no clinical improvement on broad-spectrum antimicrobials, suggestive of a non-infectious etiology of his rash and fevers. His rash evolved into confluent red patches, and eosinophilia rose to 21%, which was concerning for a drug reaction. His RegiSCAR score was calculated to be 6, consistent with definite DRESS. Leukopenia resolved ($6.3 \times 10^3/\mu\text{L}$) 4 days after discontinuing oxacillin. His epidural abscess was ultimately treated with daptomycin, and DRESS was managed supportively with antihistamines and triamcinolone cream. CONCLUSIONS We highlight this case because of the rarity of DRESS with agranulocytosis related to oxacillin. Beta-lactam antibiotics are widely used, and while DRESS is an uncommon condition, clinicians should consider this diagnosis when managing patients with fevers, leukopenia, and rash.

Drug reaction with eosinophilia and systemic symptoms related to antiretroviral treatment in human immunodeficiency virus patients.

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The drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome is a life-threatening condition caused by different medications. The objective of this study was to analyze DRESS cases related to antiretroviral therapy in human immunodeficiency virus (HIV) patients. Systematic review of DRESS suspected cases in HIV patients associated to antiretrovirals published between January 1998 and April 2017. The registry of the severe cutaneous adverse reactions score was used to classify each report as a "definitive," "probable," "possible," or "no" DRESS case. Clinical characteristics, management, and outcomes were evaluated. Thirty-five case reports were analyzed involving 5 antiretrovirals: Abacavir in 10 (28.6%) cases, efavirenz in 6 (17.1%), nevirapine in 12 (34.3%), raltegravir in 6 (17.1%), and tenofovir in 1 (2.9%). Mean age of the patients was 40 ± 13 years, 65% of which were male. A total of 57.1% reports were classified as having a "definitive-probable" DRESS case. Management was based on withdrawal of the causal antiretroviral and corticosteroids in 68.6% of the cases. None of the patients died. Treatment with nevirapine or raltegravir, the longer onset of symptoms and the presence of lymphadenopathy, eosinophilia, liver involvement, and a longer time for clinical resolution were more frequent among "definitive-probable" DRESS cases. A DRESS syndrome has to be suspected in HIV patients with lymphadenopathy, eosinophilia, and liver involvement developing weeks after the initiation of nevirapine or raltegravir. Suspension of the causal antiretroviral and in most cases treatment with corticosteroids allowed adequate clinical control.